

FORM 20 — AFFIDAVIT OF CORY LEA SHEPHERD

QUEENSLAND INDUSTRIAL RELATIONS COMMISSION — Matter No. WC/2024/227
Workers' Compensation and Rehabilitation Act 2003 (Qld); Industrial Relations (Tribunals) Rules 2011

BETWEEN: CORY LEA SHEPHERD — **Appellant** **AND:** WORKERS' COMPENSATION
REGULATOR — **Respondent** **AFFECTED NON-PARTY (Notice of Non-Party Disclosure, Form**
29): METRO SOUTH HOSPITAL AND HEALTH SERVICE

AFFIDAVIT OF CORY LEA SHEPHERD (*Form 20 — filed on behalf of the Appellant, in support
of the Application in Existing Proceedings (Form 4) under r 64G*)

I, CORY LEA SHEPHERD, of 15 Edmond Street, Coomera in the State of Queensland, Administration
Officer, do solemnly and sincerely affirm and declare:

Purpose, and the Amended 9A

1. I make this affidavit in support of my application (Form 4) in this matter and of my Notice of Non-Party Disclosure (Form 29). I refer to my Amended Statement of Facts and Contentions filed 7 April 2026 ("the Amended 9A"). The facts below are within my own knowledge except where stated otherwise, and they go to the stressors pleaded in the Amended 9A and to the relevance and necessity of the documents sought in the Form 29. I set the facts out under the same structure as the Amended 9A.

2. I am aware that the statutory exclusion under s 32(5)(a) of the *Workers' Compensation and Rehabilitation Act 2003* is for the Respondent to establish. I make this affidavit because the practical task of proving the matters pleaded rests on me, and because the documents that prove them are held by Metro South Hospital and Health Service ("MSH"), not by me.

PART A — MY POSITION

3. **Position.** I am employed by MSH as an Administration Officer, classification AO3, in Switchboard Services at Logan Hospital (organisational unit "MSH CI LOGH Switchboard Srvs"; position 32002276). I am commonly described as a Switchboard Operator.

4. **Permanent appointment.** I was converted from temporary to permanent employment with effect from 3 March 2021 under the temporary-to-permanent conversion directive. My appointment is permanent and I am classified as a Continuous Shift Worker (64 hours per fortnight on conversion;

five weeks' recreation leave). Annexed and marked "CS-1" is the myHR Movement Details record.

5. **Full-time hours.** On 31 August 2023 I applied of my own initiative to increase my hours to full time on a rotational roster. My application was approved on 27 September 2023 and I commenced full-time hours on 16 October 2023. Annexed and marked "CS-2" is my application of 31 August 2023.

PART B — MY ROLE (THE FULL PICTURE)

6. **The importance of the role.** The switchboard is the hospital's communications hub and a critical part of its patient-safety system. In an emergency (a Code Blue, a MET call, a security or clinical emergency) it is the switchboard that activates and pages the response teams, through the Spok messaging and paging system. Whether the right clinician is reached, and how quickly, depends on the switchboard and on the information in Spok being correct. The role is not peripheral administration; it is the point at which urgent help is summoned. An error or delay at the switchboard is an error or delay in the emergency response itself. The Respondent admits that "[m]aintaining accurate contact details for medical staff is a critical function of the Switchboard to ensure effective clinical handover and patient safety" (Admitted Fact: Form 24, Para 8).

7. **The systems I operate.** In my application of 31 August 2023 I recorded, in my own words, the systems I operate. I operate concurrently and in real time a number of separate systems, including the Alcatel switchboard console, the Spok messaging and paging system, Cerner, HBCIS, EDIS (the Emergency Department web database), Microsoft 365, and spreadsheets used for rostering and call records. I move between these systems continuously while answering calls.

8. **The first point of contact.** The switchboard is the first point of contact for Logan Hospital. The switchboard number is the principal published telephone number for the hospital (the first number listed for the public) and it is also the number that staff throughout the hospital use and know to call. Calls from the public and from staff alike come to me in the first instance.

9. **Volume.** I handle approximately 200 to 300 calls per shift, a substantial proportion of which come from people who are distressed, frightened, in conflict, or in crisis. I am frequently the first person a caller reaches in an urgent or emergency situation. Annexed and marked "CS-8" are the monthly statistics ("Individual monthly stats") which I maintain and author as part of my role. CS-8 is a tally of volume only — it records the number of calls I handled and emergency codes activated. It does not record which clinicians were paged, whether a page was received or acknowledged, or whether a page was misdirected; that system-level data is held only by MSH in the Spok system and I cannot produce it. Because CS-8 records emergency-code data referable to clinical events, it is provided subject to the non-publication order sought in the application; the aggregate figures stated in this affidavit may be received in open evidence.

10. **Triage.** A core part of my function is to triage and connect — directing calls ward-to-ward and from one point of urgency to another, identifying which calls cannot wait, and routing them to the correct clinician or team without delay, while continuing to answer the queue and operate the systems above.

11. **Locating emergency callers.** In an emergency, a caller is sometimes unable to tell me where they are calling from. In those circumstances I must identify the source and location of the call so that help can be directed to the right place. Any delay in doing so may delay the emergency response.

12. **Emergency codes and concurrent urgencies.** A core part of my function is the activation of emergency codes. Between 15 and 50 emergency codes are sent out through the Spok system by the switchboard over a 24-hour period, each paged simultaneously to approximately 10 clinicians who together form the responding team. These, and other urgent matters, do not arrive one at a time; during a single shift I manage a number of urgencies concurrently (code calls, emergency and security responses, and time-critical clinical calls) escalating each to the correct clinician, team or responder while continuing to manage the remaining calls in the queue.

13. **Direct relay of urgent clinical results.** A core clinical function of my role is the direct relay of urgent pathology and other clinical results between doctors (doctor-to-doctor, including to and from external doctors and facilities (for example, Hibiscus)) where any delay may directly affect a patient's treatment. I also connect and coordinate with external agencies, including the Queensland Ambulance Service and the Queensland Police Service.

14. **The criticality of that function, illustrated.** The importance of this relay function is illustrated by occasions on which a doctor holding urgent results could not be connected to the on-call clinician until I identified the difficulty at the switchboard and connected them directly. Where the on-call contact information on which the switchboard depends is not kept current, the resulting delay falls on urgent clinical communications, and it is at the switchboard (and on the operator) that the failure first appears and must be resolved in real time. I give this to illustrate the role; I do not rely on it as a stressor.

15. **The Direct Support Officer (DSO) function.** Each medical department has a Direct Support Officer ("DSO") — an administrative officer who provides direct support to that department, including in relation to its medical rostering and leave. When a department's DSO was not contactable, I performed that support function from the switchboard, in addition to my own duties (resolving conflicts in doctors' leave and rostering, escalating matters to executives, and managing intra-hospital disputes) while continuing to answer the call queue.

16. **Roster and leave administration.** My role includes administrative responsibilities carried out alongside the live switchboard work, including maintaining and updating doctor rosters and staffing numbers, and leave administration for doctors.

17. **The daily roster and the Spok dependency.** The switchboard runs the daily roster that is referred to and relied upon across the hospital. Spok pages clinicians by reference to the on-call roster, so the accuracy of that roster determines whether an emergency page reaches the right, available clinician.

18. **Weekend coverage.** On weekends, the management structure that operates during business hours is not present. During those periods the switchboard manages doctor leave, updates rosters, and carries out whatever else is required, in effect performing functions that would otherwise be performed by management.

19. **Complaints.** Complaints come directly to the switchboard (by telephone and to the switchboard email) and frequently take the form of accusations made directly to me. I am able to action and resolve a substantial proportion of them myself. Others fall outside the authority of my role and must await my manager or be routed to the dedicated complaints area, but in each case they first come to the switchboard and are dealt with by me. Where a complaint required my manager to action it, my ability to resolve it was limited by whether and when my manager was available.

20. **Complaints about my own manager.** My manager was frequently not contactable or reachable, and did not action the complaints or issues that were raised with her. Two consequences followed. First, complaints about my own manager (her unavailability, her responses, and her inaction) came directly to the switchboard, so that I was receiving and managing complaints concerning the very person to whom I would otherwise escalate them, and at times complaints that concerned the switchboard itself. Second, the matters she did not action generated further complaints, which then fell to me to absorb and manage. These arose because my manager was not present in the fast-paced environment in which the complaints were generated, and because there was no one else to whom they could be directed. By way of example, in my email of 15 May 2024 (Exhibit "CS-5") I recorded that clinicians and staff from other departments had sought updates on pager statuses and other requests which were, in the words I used at the time, "often left unresolved due to ambiguity around when [Ms Taylor] is available."

21. **My vantage point.** Because the switchboard is the central point to which complaints are directed, I am positioned to observe directly, and over time, both the volume and the subject-matter of the complaints the hospital receives.

22. **The increase over the period.** From that vantage point I observed that, over the period pleaded in the Amended 9A, the volume and the subject-matter of the unresolved complaints and requests coming to the switchboard (including those concerning my manager's availability and the currency of the on-call roster) increased. The records that would establish the full extent of that increase are held by MSH (Items 3(a) and 3(b) of the Form 29), and I cannot produce them.

23. **The baseline (the standing condition of the role).** The defining condition of my role is this:

from the switchboard I have a vantage point over everything that goes wrong in the hospital, and I am accused of, and held responsible for, much of it (including matters properly the responsibility of others (the medical departments' Direct Support Officers, doctor availability, the updating of Spok, and decisions made at executive level)) regardless of where the fault actually lies. The volume is constant: approximately 200 to 300 calls per shift, a substantial proportion from people who are distressed, frightened, in conflict or in crisis, while I maintain the systems and the roster on which the hospital's emergency paging depends.

24. The patient-safety function was my role. The patient-safety function of the hospital's emergency communications was not separate from my role; it was my role. I was the operator through whom emergency codes were activated and urgent clinical and pathology results were relayed doctor-to-doctor. The conditions imposed on the Switchboard were therefore not conditions affecting an administrative role at one remove from patient safety — they were conditions imposed at the very point on which patient safety depended, and on the person holding it.

25. That standing condition (bearing the patient-safety load of a hospital's communications, at constant volume, while absorbing the blame for failures across the whole system, and while the support above me was absent or unmaintained) is the condition in which I performed every duty described above, and against which the management conduct described below must be assessed.

PART C — STRESSOR 1: COURSE OF MANAGEMENT CONDUCT (Amended 9A, Stressor 1)

26. Erratic presence and unilateral directives without consultation [1(a)]. Throughout 2023 and 2024 my Line Manager, Ms Chloe Donovan-Taylor ("Ms Taylor"), maintained an erratic physical presence and imposed unilateral directives without consultation. The objective record of her working presence is held in MSH's security-access system (Item 9 of the Form 29).

27. The MASPER directive [1(a)]. The misrouted-call issues raised by the MASPER registrar and the medical department in May 2024 were, in substance, a problem of the roster not reflecting doctors' actual availability — a doctor, roster and coordination problem, not an error of the operators. Ms Taylor's own email of 9 May 2024 records that a doctor's hours were "not provided on the rosters" and that the contact number was "switched off." Rather than consult the switchboard (which could have identified the cause) or check the roster herself, she directed a change to the switchboard's call-handling process. Annexed and marked "CS-3" is that email chain.

28. I raised the absence of consultation from the outset [1(a)]. On 16 June 2023 I wrote raising, among other workplace issues, "No consultation of the team members or the department when making major changes such as additional shift... commence/finish [times]." Annexed and marked "CS-4". I raised it again on 15 May 2024, and the Director, Ms Tammy Reese, asked me on 21 May

2024 to identify the directives about which I had consultation concerns. Annexed and marked "CS-5".

29. Roster/Spok errors and resulting delay [1(a)]. When the on-call roster in Spok was not kept up to date, emergency and urgent pages were directed to the wrong or an unavailable clinician. Whether a page reached the correct clinician, was received or acknowledged, or was misdirected is recorded in MSH's Spok system; that record is held only by MSH and I cannot produce it — my own statistics record volume only. These errors accumulated at the switchboard and fell to me to identify and work around in real time. I raised them by complaint; to my knowledge and observation, no change to operating procedures, no investigation, and no remedial or safety action was taken in response (a matter MSH has since confirmed in its letter of 5 June 2026 (ref K-LM26/729), which is already before the Commission, stating that "there have been no 'consequential' changes to operating procedures over the period requested"); and the failure to address them resulted in direct delay, including to urgent clinical communications.

30. The vocabulary of the complaints [1(a)]. The complaints about misdirected calls use a recognisable and recurring operational vocabulary — that a doctor or clinician was paged or directed to the "wrong ward", the "wrong location", the "wrong team" or the "wrong doctor"; that a page was "misdirected" or that there was a "pager error"; or that the "list", "directory" or "roster" was "out of date" and needed "updating". These are the actual words in which the complaints are made.

31. Destruction of WHS records [1(b)]. On 6 June 2023 I recorded a critical operational instruction in the communication book regarding the need to verify and update doctors' on-call contact details. On that day my Line Manager, Ms Taylor, removed the page containing my handwritten entry from the Communication Book (Admitted Fact: Form 24, Para 6); the removed entry concerned the verifying and updating of doctors' contact details (Admitted Fact: Form 24, Para 7).

32. Grievance and its handling [1(c)]. On 7 August 2023 I submitted a grievance regarding Ms Taylor's conduct, raising WHS concerns about unsafe rostering and fatigue. My grievance was not upheld and I was directed to continue reporting to Ms Taylor. In her email of 7 August 2023 Ms Reese acknowledged that "there was a rostering error that was accidentally made by Chloe with regards to night shifts" (Admitted Fact: Form 24, Para 5).

33. COVID special pandemic leave obstruction [1(d)]. My application for COVID special pandemic leave (Process Reference 15480560) had to be submitted four times before it was approved — once by text message to my Line Manager's mobile and three times through myHR. On myHR the application was declined twice by my Line Manager, Ms Taylor, and was ultimately approved on the same supporting evidence I had already provided. The myHR records and audit trail recording those submissions and declines are the material the subject of Item 11 of the Form 29, disclosed by MSH with its letter of 5 June 2026 (see paragraph 51); I refer to that disclosure and do not annex it.

34. Public Interest Disclosure [1(e)]. On 13 May 2024 I made a complaint concerning Ms Taylor

and Ms Reese, which the Ethical Standards Unit determined on 24 December 2024 to constitute a Public Interest Disclosure (Admitted Fact: Form 24, Para 20); the determination bears reference PID 24-ESU-1130. I do not set out the content of that disclosure in this affidavit.

35. Direction to retract within 48 hours [1(f)]. On 15 May 2024, within 48 hours of the matter referred to in the preceding paragraph, the Director, Ms Reese, directed me to retract an email I had sent inquiring about office hours (Exhibit "CS-5") (Admitted Fact: Form 24, Para 21). A substantively similar email sent by Ms Taylor on 9 May 2024 about another staff member's hours attracted no such direction (Admitted Fact: Form 24, Para 22).

36. Suppression of union representation [1(g)]. I notified management of my intention to become the switchboard union delegate on 11 August 2023 (Admitted Fact: Form 24, Para 18). My intention was also recorded in my full-time application of 31 August 2023 and expressed directly to Ms Taylor. On 18 May 2024 I sent a request regarding union representation to the Together union (Ms Heather Hayes), and on 20 May 2024 Together confirmed it had "passed [my] correspondence on to [my] Health team." Annexed and marked "CS-6". The Union Encouragement Policy (QH-POL-248) requires managers to take a "positive, supportive role" (Admitted Fact: Form 24, Para 17); management failed to facilitate the delegate appointment for a period exceeding 9 months (Admitted Fact: Form 24, Para 19), leaving the department without delegate representation while I was disputing unsafe rostering and fatigue practices.

PART D — STRESSOR 2: REMUNERATION (Amended 9A, Stressor 2)

37. Underpayment and the AVAC delay. Between February and April 2024 I was not paid my correct entitlements. On 3 May 2024 Queensland Health Payroll (Payroll Officer Elaine Grant) instructed Ms Taylor to "submit an AVAC to correct these shifts" (Admitted Fact: Form 24, Para 40); the AVAC was not submitted until 28 May 2024, a 25-day delay (Admitted Fact: Form 24, Para 41).

PART E — STRESSOR 3: STATUTORY FATIGUE BREACH (Amended 9A, Stressor 3)

38. The 7-hour break. On 17–18 March 2024 I was rostered to work consecutive shifts ending at 23:00 and recommencing at 06:00 — a 7-hour break (Admitted Fact: Form 24, Para 1), against the 10-hour minimum (or 8 hours by written agreement, of which there was none) mandated by the Award and the FRMS (Admitted Fact: Form 24, Paras 3–4). Factoring my established commute, the rest opportunity available to me was substantially less than that 7-hour gap.

39. Coordinating emergency codes under exhaustion. Under that exhaustion I was required to manage life-safety emergency codes and doctor pages through the Spok system — the very function

described in paragraph 6 above. Being required to control emergency clinical coordination in that state, after I had previously raised fatigue concerns, is the conduct pleaded at Stressor 3.

40. **Fatigue-leave refusal.** When I required recovery time, Ms Taylor applied a superseded 2020 casual agreement to deny paid fatigue leave, requiring me to exhaust my own personal leave on 19 March 2024.

41. **The Review Officer's findings.** In the Review Decision dated 24 October 2024 the Regulator's Review Officer found that rostering me with the 7-hour break "amounted to unreasonable management action", and recorded that the Regulator was "satisfied [my] employment was a significant contributing factor to the psychological injury". The contents of that Review Decision are admitted (Admitted Fact: Form 24, Para 34).

PART F — POST-INJURY AGGRAVATION (Amended 9A)

42. **Meetings while on certified leave.** After the onset of my injury, and while I was on certified leave, I was required to attend meetings convened by management on 12 and 16 July 2024.

43. **Termination and reinstatement.** On 8 October 2024 my employment was terminated on the basis of "abandonment of employment." I applied for reinstatement (matter TD/2024/110) and was reinstated.

44. **Medical records obtained without a Notice served on me.** In the course of the proceeding, the Respondent obtained my medical records under a Notice of Non-Party Disclosure (Form 29) that was never served on me; the Respondent admits that no Form 29 was served on me and relies on s 64D (Admitted Fact: Form 24, Para 25).

PART G — MY INJURY AND CURRENT CAPACITY

45. **Injury and current capacity.** As pleaded in the Amended 9A, I suffered the onset of a psychiatric injury (Major Depressive Disorder with Anxious Distress) on 18 June 2024. I returned to work on medical advice and am currently working at 0.5 FTE, and I remain unable to perform the full duties of the role described above. Annexed and marked "CS-7" are my medical certificates. The diagnosis, onset and causation of my injury are addressed in the medical evidence and in the Amended 9A, and I do not repeat that evidence here.

PART H — THE DOCUMENTS, AND WHY DISCLOSURE IS NECESSARY

46. The matters in Parts C–F are recorded in documents held by MSH — its email system, its identity-management and security-access systems, its meeting and governance records, and its roster,

payroll and HR systems. I am able to produce individual examples (such as those annexed), but I cannot export the complete record. I do not seek the review or reading of any email; I seek only the native electronic export of the messages returned by the keyword filters described in the Form 29 (identified custodians, date-ranges and phrase-families), which can be produced by MSH from its systems at minimal cost.

47. Because my injury developed over the course of the management conduct described above, the matter the documents go to is the cumulative volume and frequency of that conduct across the whole pleaded period — including the full corpus of directives sent from my Line Manager to the Switchboard. Individual examples show that the conduct occurred; only the complete export shows how much of it the role endured over the course of conduct.

48. Because the practical burden of proving these matters rests on me, and because I cannot prove their full extent without MSH's records, the production sought in the Form 29 is necessary. Where a document the Form 29 seeks does not exist, I ask that its non-existence be verified on oath.

49. On 5 June 2026 MSH responded to the Form 29 by letter signed by its Chief Executive (ref K-LM26/729). That letter is already before the Commission, and I therefore do not annex it but refer to it and rely upon it. In that letter MSH stated, among other things: that there had been "no consequential changes to operating procedures" over the period (Item 3(c)); that fatigue risk management at the Switchboard was implemented "after 30 June 2024 in connection with an organisational change related to the reporting lines for Switchboard" (Item 5); that the SPOK records "do not exist" because "records predating [the system] upgrade were not retained," while "a spreadsheet of recorded MET calls is available for the period 17–18 March 2024" (Items 1–2); and that the text messages between me and my manager were on her personal mobile device, no Queensland Health device having been issued to her until the third quarter of 2024 (Item 14). I depose only that the letter states these matters, and I rely on it as referred to above; their significance is addressed in the application and submissions.

50. As to Items 1 and 2 (the SPOK emergency-paging records), I operate the SPOK paging system as part of my daily duties, and it is the system through which the switchboard activates and records emergency codes and pages. In its letter MSH states that those records "do not exist" because the SPOK system "was upgraded" and "records predating that upgrade were not retained," while in the same letter confirming that "a spreadsheet of recorded MET calls is available for the period 17–18 March 2024." I am not able to reconcile an upgrade of the system with the disposal of its emergency-code paging records. I therefore ask that the verification on oath I seek address, for Items 1 and 2: the date and nature of the SPOK upgrade; whether the records predating it were migrated, archived or destroyed; the basis on which they were "not retained"; and whether any backup or archive of those records exists.

51. With its letter of 5 June 2026, MSH also disclosed documents in answer to Items 6, 11, 12, 13, 15 and 16 of the Form 29. Those documents are before the Commission and I refer to them rather than annex them. They include: the Role Description for the Manager of Switchboard Services (Item 6); the HR sub-delegation instrument for paid special pandemic leave (the COVID-19 Pandemic Event Instrument) together with Pandemic Directive 01/20 and its Implementation Guideline (Item 13); my myHR leave records and AVAC history for the pandemic-leave period (Item 11); the pandemic-leave audit trail for the Logan Hospital Switchboard team (Item 12); my leave-takings report for 19 March 2024 (Item 15); and the daily staffing-variance forms for the periods 5–18 February and 19 February–3 March 2024 (Item 16).

52. On 11 June 2026 the Respondent disclosed in this proceeding email correspondence dated 15–16 May 2024 concerning my complaint and the operational and human-resources response to it — correspondence falling within the 13–31 May 2024 period that is the subject of Item 20, which MSH states in its letter "does not exist" (on the basis that the reference "PID24-ESU-1130" "commenced in November 2024"). I do not set out the content of that correspondence. I refer to that disclosure in support of the matters deposed in Parts C to F; its significance is addressed in my submissions.

53. Material has also been disclosed through the witness-conferencing process in this proceeding (the emails I have previously described as the "Disclosure from witness conferencing"). That material is confidential and is before the Commission; I refer to it and do not annex it.

AFFIRMED by the deponent CORY LEA SHEPHERD

at (place) in the State of Queensland on
..... (date) 2026

..... (signature of deponent)

Before me:

..... (signature of witness)

..... (full name and qualification of witness — Justice of the Peace /
Commissioner for Declarations / Lawyer)

(The deponent and the witness sign the foot of every page of this affidavit. Each exhibit "CS-1" to "CS-8" is to bear a Form 21 exhibit certificate completed and signed by the witness.)

EXHIBIT INDEX (Form 21 certificate on each)

Mark	Document	Para	Note
CS-1	myHR Movement Details: permanent AO3, Switchboard, Logan Hospital, effective 3 March 2021 (Continuous Shift Worker)	4	
CS-2	Application dated 31 August 2023 to increase to full time (approved 27 Sep 2023; commenced 16 Oct 2023)	5	
CS-3	Email chain re the MASPER directive, including the email of 9 May 2024	27	
CS-4	Email dated 16 June 2023 raising the absence of consultation on shift commence/finish times	28	
CS-5	Correspondence dated 15 and 21 May 2024 re consultation on directives (Director, Ms Tammy Reese)	20, 28, 35	15–21 May portion only: excise the 11 July 2024 email (pp 7–8)
CS-6	Correspondence 18–20 May 2024: the Appellant's 18 May request to the Together union (Ms Heather Hayes) and the union's 20 May confirmation it passed the correspondence to the Health team	36	
CS-7	Medical certificates: MDD with Anxious Distress; onset 18 June 2024; currently 0.5 FTE (to encompass the Psychiatric Report of Dr Ravikumar Krishnaiah and the Work Capacity Certificates of Dr Peter Hawes)	45	s 580 non-publication
CS-8	Monthly call statistics ("Individual monthly stats"), maintained and authored by the deponent	9	emergency-code data — s 580

Referred to, not annexed (already before the Commission / disclosed in the proceeding): the MSH letter of 5 June 2026 (ref K-LM26/729) and the documents disclosed with it (Items 6, 11, 12, 13, 15, 16) — at ¶29, ¶49 and ¶51; and the Respondent's disclosure of 11 June 2026 (the 15–16 May 2024 correspondence concerning the complaint) — at ¶52.

FILED BY: Cory Lea Shepherd, Appellant (Self-Represented) — 15 Edmond Street, Coomera QLD

This affidavit comprises numbered paragraphs 1 to 53 and exhibits CS-1 to CS-8, set out under the same structure as the Amended 9A. The MSH letter of 5 June 2026 — and the documents disclosed with it, including the Item 11 myHR leave records and AVAC history — are referred to, not annexed (already before the Commission). The Respondent's disclosure of 11 June 2026 (the 15–16 May 2024 correspondence) is likewise referred to, not annexed.

NOT PART OF THE AFFIDAVIT — PRE-LODGEMENT CHECKLIST (working aid; remove before swearing)

Completed at the moment of swearing (do not pre-fill):

- ☐ Date and place in the jurat
- ☐ Deponent + witness signature at the foot of every page
- ☐ Form 21 certificate signed by the witness on each exhibit CS-1 ... CS-8
- ☐ Medical certificates physically annexed as CS-7

Verify before lodging:

- ☒ Amended 9A filing date — resolved. The Amended 9A was lodged across 7–8 April 2026; "filed 7 April 2026" stands.
- ☐ New admitted-fact citations — confirm against the Respondent's Form 24 (SOFC) PDF before swearing: ¶34 (PID, Para 20), ¶35 (retraction, Para 21), ¶44 (medical records without a Notice, Para 25). Existing citations Paras 1, 3, 5, 14, 17, 18, 37, 40, 41 also to be re-checked.
- ☐ New facts to confirm: ¶33 (four submissions, once by text, three via myHR; two declines by Ms Taylor; her lack of delegated authority — confirm against the Item 11 myHR audit trail and the Item 13 HR delegation instrument; confirm the delegation date: the Instrument states effective 5 December 2022, while MSH's letter referred to it as "signed 23 November 2022"); ¶42 (meetings 12 & 16 July 2024 while on certified leave); ¶43 (8 Oct 2024 abandonment termination and reinstatement, TD/2024/110).
- ☐ ¶40 — **contested characterisation on oath**. "a superseded 2020 casual agreement" is a characterisation the Respondent contests (it relies on an 8-hour written agreement signed June 2020). Either keep only if you can prove the 2020 agreement was superseded, or render it factually ("Ms Taylor relied on a 2020 agreement to deny paid fatigue leave, requiring me to exhaust my own personal leave on 19 March 2024"). Note: a 7-hour break breaches even an 8-hour agreement, so this paragraph is not load-bearing.
- ☐ CS-5 must be curated — annex only the 15–21 May 2024 exchange (pp 1–6). Excise the 11

July 2024 email (pp 7–8): fraud/corruption allegations and personal/family detail, barred and prejudicial.

- ☐ ¶14 pathologist episode — confirm date (~early July 2024, post-injury; illustrative only). No patient/pathologist/registrar named. Cut if not certain.
- ☐ CS-7 to encompass the Krishnaiah psychiatric report and the Hawes Work Capacity Certificates (both for s 580 sealing).
- ☐ Bundle exceeds 30 pages → lodge in hard copy; serve on the Respondent (OIR) and on MSH (affected non-party).